NCCP 00319 · clinician draft

Carboplatin + Oral Etoposide

Which treatment this guide covers This guide covers carboplatin plus oral etoposide for extensive-disease small-cell lung cancer in people unsuitable for cisplatin-based treatment. It does not cover an intravenous etoposide schedule or an added immunotherapy medicine. If the prescription differs, ask for its matching guide and consent discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin AUC5 plus oral etoposide

Active medicines: carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before SACT on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V8 row This is the dedicated carboplatin row. Reconcile with the full multi-day plan; no duplicate steroid courses.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.

Additional phase-specific support

- Dedicated carboplatinAUC≥4NCISV8row; routine olanzapine is not included in this row.
- Only Day 1contains high-risk carboplatin. Days 2–3oraletop are minimal–low risk and need their own daily review within one reconciled plan.
- NCIS multi-day principle:appropriate prevention on each treatment day and two days afterwards. Define the Day 4–5/ rescue plan without inventing extension of every high-risk medicine.
- Metoclopramide row does not define end day or PRN qualifier; prescriber/pharmacy must set duration and suitability.
- Check QT risk for ondansetron and all interactions; local policy and individual prescriptions apply.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00319 v5 · pNCCP 00319pp1,4–5;NCISV8Table1p4andmulti-daynotep5 · NCCP NCIS Medical Oncology antiemetics V8, Table 1

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required